

Healthcare Utilisation & Clinical Quality Analysis Report (2017–2019)

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1. Executive Summary

This report provides an overview of healthcare utilisation, financial performance, emergency department activity, and hypertension control across a three-year period. The findings indicate a system that is stable, efficient, and clinically well-managed. Utilisation declined steadily, cost structures remained consistent, emergency demand was low, and hypertension control rates were exceptionally strong.

2. Dataset Overview

The analysis draws on encounter-level and observation-level data, including:

- Encounter dates and care group classifications
- Total and average costs
- Emergency department utilisation patterns
- Systolic and diastolic blood pressure readings
- Patient-level identifiers

This dataset supports both operational and clinical quality insights.

3. Pillar-by-Pillar Insights

3.1 Utilisation Analysis

- Total encounters declined each year from 2017 to 2019.
- Average cost per encounter remained stable, indicating consistent case mix and pricing.
- Scheduled and preventative/community care accounted for the majority of activity and cost.
- Acute care represented the smallest share of encounters and spending.

Interpretation: The system became less busy but not more expensive, suggesting reduced demand or improved efficiency rather than operational strain.

3.2 Financial Analysis

- Total costs decreased in line with declining utilisation.
- Average cost per encounter was nearly identical across all care groups and all years.
- No evidence of inflationary drift or rising unit costs.

Interpretation: Financial performance was stable and predictable, with cost changes driven entirely by encounter volume rather than increased resource use.

3.3 Emergency Department Analysis

- ED encounters represented only 4–5% of total activity.
- Most patients visited the ED once per year, indicating low utilisation intensity.
- Total ED spending decreased after 2017.
- Average ED cost per encounter remained constant across all years.

Interpretation: Emergency demand was low, stable, and well-controlled, with no signs of overuse or cost escalation.

3.4 Hypertension Monitoring & Control

- 346 patients had at least one BP reading recorded.
- Only 20 patients had any episode of uncontrolled BP.
- This results in a BP control rate of approximately 94% across the population.
- BP monitoring coverage was strong and consistent.

Interpretation: Hypertension management was highly effective, with excellent control rates and reliable monitoring practices.

4. Key Metrics Summary

Total Patients with BP Recorded: 346

Patients with Uncontrolled BP: 20

BP Control Rate: ~94%

ED Share of Total Activity: ~4–5%

ED Utilisation Intensity: Low (mostly single-visit patients)

Cost Trend: Decreasing (volume-driven)

Average Cost per Encounter: Stable across all years

5. Recommendations

- Maintain current hypertension monitoring protocols.
- Investigate drivers of declining utilisation.
- Continue monitoring ED utilisation intensity.
- Leverage cost stability for long-term planning.

6. Limitations

- The dataset does not include clinical outcomes beyond BP control.
- Encounter severity and diagnosis information were not available.
- Social determinants of health were not included, limiting contextual interpretation.

7. Conclusion

From 2017 to 2019, the healthcare system demonstrated declining utilisation, stable cost structures, low emergency dependence, and excellent hypertension control. These trends reflect a well-managed, efficient, and clinically stable environment, with no signs of operational strain or deteriorating patient outcomes.